

Evaluation of the Drug Use Disorders Identification Test (DUDIT) in Criminal Justice and Detoxification Settings and in a Swedish Population Sample

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Key Words

Screening, DUDIT evaluation · Substance abuse · DUDIT, validity · DUDIT, reliability · Population norms

Abstract

Psychometric properties of the 11-item self-report Drug Use Disorders Identification Test (DUDIT) were evaluated in a sample of heavy drug users from prison, probation, and inpatient detoxification settings, and in a general Swedish population sample. In the drug user sample, the DUDIT predicted drug dependence with a sensitivity of 90% for both DSM-4 and ICD-10 and a respective specificity of 78 and 88%. Reliability according to Cronbach's α coefficient was 0.80. In the population sample, 3.1% scored positive on the DUDIT; T-score values are suggested. The DUDIT screens effectively for drug-related problems in clinically selected groups and may prove useful in the context of public health surveys.

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A simple self-report screening instrument focusing on current drug-related problems and facilitating comparison between different groups has been lacking in the literature. Practitioners and patients in criminal justice, addiction treatment and psychiatric settings need to gain a

quick impression of the extent of problematic drug use prior to further assessment. Another crucial reason for screening is that the use of one or more illicit drugs is often a complicating factor in treatment of primary presenting problems in settings outside addiction. We developed the Drug Use Disorders Identification Test (DUDIT) following an extensive literature survey and think-aloud testing of three preliminary versions [unpubl. data], to offer optimal simplicity in the screening process. The final 11 items in the DUDIT were chosen to give information on the level of drug intake and fulfilment of selected criteria for substance abuse/harmful use and dependence according to the ICD-10 and DSM-4 diagnostic systems. The DUDIT was developed and tested in Sweden, with the intention of expanding trials in Europe and elsewhere once psychometric data for at least one country were established.

Available screening instruments vary in terms of focus and score utility. The self-report 10-item DAST [1] identifies severity of current drug problems without specifying target groups and the 6-item UNCOPE [2] detects current alcohol and drug dependence in the narrow target group of arrestees. Brief screening interview schedules such as the CAGE-AID [3] or the ASSIST [4] target primary care patients and focus on detection of hazardous and harmful alcohol and other substance use, current for the CAGE-AID and both current and lifetime for the ASSIST.

A limitation that the DAST, UNCOPE, CAGE-AID and ASSIST instruments share is that they offer only a dichotomous yes/no response option, limiting information on the behavioural frequency of each item at the stage of initial assessment.

The DUDIT is a self-report instrument intended for use together with the Alcohol Use Disorders Identification Test (AUDIT [see 5]). Both instruments yield scores on a continuous interval scale and can easily be used in criminal justice, addiction treatment and psychiatric settings. The DUDIT is presented on one sheet, in a user-friendly graphic design with the 11 items on the front page. The back page shows a list of common illicit drugs in seven categories, followed by commonly abused sedatives, hypnotics and analgesics (see Annex 1).¹

This study evaluates the DUDIT in terms of its psychometric properties in two Swedish samples. In the first sample we tested the DUDIT for reliability and concurrent validity using SCAN diagnostic interviews with drug using respondents from prison and probation, and from addiction inpatient detoxification settings. Secondly, we tested the DUDIT in a random sample from the general population for descriptive statistics and reference values.

Method

Drug User Sample Participants

In order to obtain a high prevalence sample, we recruited 160 interviewees from in-patient Stockholm Addiction Center (SAC) drug detoxification units at Danderyd and Huddinge hospitals and from among remand prison detainees, prison inmates and probation clients in the cities of Gothenburg and Stockholm (Kriminalvårdsstyrelsen [Swedish Prison and Probation Administration] – KVS). Only interviewees with known drug use were interviewed.

Measures and Scoring

Screening was performed with the DUDIT. The first 9 items with five response alternatives each were coded 0, 1, 2, 3, or 4, and items 10 and 11 with three response alternatives were coded 0, 2 or 4, giving a maximum score of 44 points.² For validity diagnosis of harmful use/substance abuse and dependence according to both ICD-10 and DSM-IV, the Swedish translation of Chapter 12 of the Schedules for Clinical Assessment in Neuropsychiatry (SCAN), Version 2.1 [6, 7], was chosen as a 'gold standard' interview schedule. Diagnoses were obtained using computer algorithms [8].

¹Annex shows an untested preliminary English version of the DUDIT. Researchers or clinicians wishing to use the DUDIT in English should contact the authors regarding an authorized translation.

²The coding scheme follows that of the AUDIT in order to facilitate combined use of the two instruments. The maximum score for the AUDIT is 40 points.

Procedure

Interviewees were informed that the interview would take about 1 h and that they would be remunerated with a gift worth approximately EUR 4. At the detoxification units, a registered nurse and a social worker carried out 105 of the interviews. At the prison and probation sites, probation officers carried out 40 of the interviews and 15 were completed by the first author.

Population Sample Participants

A random sample of 1,500 persons from 16 years and up was selected from the Swedish population registry. An independent consulting company (Schlumberger SEMA) conducted the randomization procedure using a random number-based selection among all Swedish residents showing a registered address in the population register.

Procedure

A cover letter from Karolinska Institutet described the study. Respondents were also informed that they could receive a gift check worth about EUR 5.50 if they communicated their questionnaire identification number (thus revealing their identity). The first 1,500 questionnaires were sent out in May of 2002. Two reminders were sent to non-responders, in June and August of the same year.

Analysis for Both Samples

For the drug user sample, analysis included inter-item and item-total correlations, factor analysis, reliability, and validity based on receiver operating characteristic (ROC) curves. The population sample was analyzed descriptively and t-score transformations were performed, in addition to reliability and factor analyses. SPSS version 11.0 was used for all analyses.

Ethics for Both Samples

The Regional Ethical Committee of Karolinska Institutet approved testing of the instrument among SAC respondents for the drug use sample (permit No. 01-228, 2001-09-19) and for the population sample (permit No. 01-228, 2002-04-16). The Linköping University Regional Ethical Committee approved interviews with Gothenburg and Stockholm respondents under the jurisdiction of KVS (permit No. 01-255, 2001-06-21).

Results

Drug User Sample

Interviewees numbered 160. Six men were excluded from the final sample, 2 because they were in a state of clouded consciousness at the time of the interview and 4 because of missing values. The final sample thus consisted of 154 respondents, of whom 83% (128) were men and 17% (26) were women. The age range was 18–74 years with a mean age of 34 years. Most interviewees (81%) were interviewed in Stockholm and the remaining 19% in Gothenburg. According to DSM-4 diagnoses, 94% of the respondents were dependent on at least one drug. Twenty-five percent were dependent only on one drug, 60% were dependent on two or three drugs and 8% were

Table 1. Inter-item and item-total correlations for the DUDIT in a sample of drug users (n = 154). Correlations over 0.30, all significant at $p < 0.01$, are in boldface type

DUDIT items	1	2	3	4	5	6	7	8	9	10	11	Item total
1 How often do you use drugs other than alcohol?	1											0.68
2 Do you use more than one type of drug on the same occasion?	0.48	1										0.56
3 How many times a day do you take drugs on a typical day when you use drugs?	0.21	0.28	1									0.38
4 How often are you influenced heavily by drugs?	0.25	0.16	0.32	1								0.52
5 Over the past year, have you felt that your longing for drugs was so strong that you could not resist it?	0.46	0.22	0.20	0.37	1							0.72
6 Has it happened, over the past year, that you have not been able to stop taking drugs once you started?	0.43	0.21	0.15	0.33	0.67	1						0.68
7 How often over the past year have you taken drugs and then neglected to do something you should have done?	0.35	0.13	0.09	0.17	0.44	0.35	1					0.57
8 How often over the past year have you needed to take a drug the morning after heavy use the day before?	0.49	0.30	0.17	0.35	0.63	0.55	0.32	1				0.73
9 How often over the past year have you had guilt feelings or a bad conscience because you used drugs?	0.21	0.17	0.01	0.16	0.26	0.21	0.37	0.34	1			0.55
10 Have you or anyone else been hurt (mentally or physically) because you used drugs?	0.28	0.28	0.10	0.08	0.12	0.15	0.22	0.21	0.36	1		0.50
11 Has a relative or a friend, a doctor or a nurse, or anyone else, been worried about your drug use or said to you that you should stop using drugs?	0.24	0.36	0.11	0.11	0.20	0.21	0.01	0.27	0.20	0.25	1	0.45

dependent on four to six drugs. The prevalence of dependence on various drugs varied: 60% of the interviewees were dependent on opiates, 53% on somniferents, 40% on cannabis, 34% on amphetamines, 5% on cocaine, 4% on hallucinogens and 3% on other drugs such as GHB. Substance abuse according to DSM-4 was diagnosed for 16% of the sample on cannabis, 14% on amphetamines, 12% on hypnotic and anxiolytics, 8% on cocaine, 6% on hallucinogens and 4% for opiates. Two respondents did not fulfil any diagnostic criteria for substance abuse or dependence.

Missing Values and Item Characteristics

Options for handling missing values include elimination or estimation by educated guess, means calculation based on existing data or prediction by regression [9]. The respondents in this sample were already identified as drug users and it was assumed that they had no immediate gain from not replying to an item. For this reason, we decided to interpret missing values as intended null values for all cases in the drug user sample.³

³ In a lower prevalence sample with unidentified drug users (e.g., psychiatric patients) a missing value could be interpreted as resistance to admitting drug use, in which case we would have handled missing values differently.

Analysis of the DUDIT item distributions showed moderate negative skewness (between -0.74 and -3.4) for all variables except item 3, which was slightly positively skewed (0.34). The kurtosis varied between -0.81 (item 9) and 10.6 (item 11). An outlier analysis did not reveal any cases that warranted exclusion.

Means and Group Comparisons

The DUDIT mean score for the drug user sample was 33.2 points (SD = 8.0). Scores were compared by gender, age category (≤ 30 , 31–44, and 45+ years), institutional category (KVS/SAC), and city of residence. A significant difference was found between institutional categories ($F(1,152) = 11.88$, $p < 0.001$), with respondents from the KVS showing lower scores (mean = 30.2, SD = 8.9, $n = 52$) than respondents from the SAC (mean = 34.8, SD = 7.0, $n = 102$). A tendency towards a difference between men and women was noted ($F(1,152) = 3.57$, $p < 0.10$), where men had lower mean total sums (mean = 32.7, SD = 8.1, $n = 128$) than women (mean = 35.9, SD = 6.6, $n = 26$). No other significant differences were found.

Correlations

Pearson bivariate correlations were calculated for all eleven items. Table 1 shows inter-item and item-total

correlations. Item-total correlations ranged between 0.38 and 0.73 and were all significant at the $p < 0.01$ level.

Factor Analysis

Principal components extraction with Varimax rotation first yielded an unsatisfactory three-factor solution with multiple factor loadings for four DUDIT items. Principal factor analysis with oblique rotation yielded three factors labelled dependence (items 1, 4, 5, 6, 7 and 8; factor loadings 0.48, 0.56, 0.82, 0.60, 0.74 and 0.90 respectively), drug-related problems (items 2, 10 and 11; 0.76, 0.70 and 0.67) and intensity of use (items 3 and 9; 0.66 and -0.54).⁴ The factor correlation matrix for the raw scores showed a correlation of 0.35 between factors 1 and 2, thus supporting the choice of oblique rotation as adequate for the DUDIT data in this sample.

Reliability and Validity

Reliability analysis of the DUDIT yielded a Cronbach α value of 0.80 for the total score, a value that is well above the standard definition of reliability coefficients above 0.70 as good [10]. The low number of items in the DUDIT also suggests that this reliability coefficient is fully adequate [11].

For validity analysis, SCAN diagnoses were recoded into four dichotomous yes/no variables indicating harmful use/abuse or dependence according to ICD-10 or DSM-4. Almost all the respondents had a diagnosis of drug dependence according to ICD-10 (95%, $n = 146$) and according to DSM-4 (94%, $n = 145$). These diagnoses concerned one to six drug categories. A harmful use diagnosis for at least one drug according to ICD-10 was obtained for 16% ($n = 24$) of the respondents, while a diagnosis of substance abuse according to DSM-4 was obtained for 42% ($n = 64$) of the respondents. It is important to note that the present sample had a very high prevalence of drug dependence, implying that most diagnoses for harmful use or substance abuse referred to *secondary* drug use.

ROC curves were calculated in order to identify cut-off values for maximum sensitivity (i.e., the proportion of respondents who were correctly identified by the DUDIT as dependent on drugs according to the DSM-4 or ICD-10) and specificity (i.e., the proportion who were correctly identified as *not* dependent on drugs). For the ICD-10 dependence syndrome, the area under the curve

was 0.95. At a DUDIT cut-off score of 25 points, sensitivity was 90% and specificity was 88%, both fully acceptable values. For the DSM-4 dependence diagnosis, the area under the curve was 0.94 and a cut-off score of 25 points yielded fully acceptable 90% sensitivity but less than optimal specificity of 78%. For the diagnoses of harmful use and substance abuse the area under the curve was 0.56 and 0.50 respectively, indicating that the DUDIT did not predict these diagnoses better than chance.

Population Sample

Of the 1,500 questionnaires sent out, 18 were sent back due to a change of address and 2 were returned because the intended respondents had died. The final sample thus consisted of 1,480 persons. The initial response rate was 57% of the sample (848 respondents). Five individuals returned their questionnaires unanswered. Following the first reminder, an additional 217 questionnaires arrived, raising the total response rate to 72%. After the second reminder, 87 questionnaires arrived, raising the final response rate to 77.8%, or 1,152 persons. Of these, 38 did not answer all the items, leaving 1,109 complete forms of the 1,480 forms sent out, a net response rate of 75%. Among the 1,109 respondents with completed forms, 3% ($n = 33$) had a DUDIT total score of 1 point or more. The remaining 97% had a DUDIT total score of zero. Total DUDIT scores ranged between 0 and 25. Eight percent of the respondents requested remuneration.

Dropout Analysis

The percentage of respondents with a total DUDIT score of 1 point or more increased following the first and second reminders. While only 2% among the initial set of respondents showed positive DUDIT scores, 5.1% of respondents following the first reminder showed positive DUDIT scores. The difference was even more notable among respondents following the second reminder, with 9.1% scoring positively ($\chi^2 = 15.07$, d.f. = 2, $p < 0.001$). This suggests that persons with problematic drug use delayed in responding to the questionnaire, which may also mean that many active drug users did not respond at all.

Age and Gender

The age range was between 16 and 85 years, initially divided into seven 10-year categories (16–25, 26–35, 36–45, 46–55, 56–65, 66–75 and 76–85 years). χ^2 analyses with these age group categories and a dichotomous dependent variable indicating either a zero or positive score

⁴Variable transformation by a reflected square root procedure [9] did not radically change the results of the factor analysis, thus suggesting a stable factor structure.

Table 2. T-score values for the DUDIT, men and women from the general Swedish population (n = 1,109)

DUDIT score	T-scores for men				T-scores for women			
	16–25 years	26–45 years	45 years or older	all ages	16–5 years	26–5 years	45 years or older	all ages
0	47.93	48.22	49.07	48.47	48.75	48.89	49.10	49.06
1	51.21	51.67	54.97	52.55	59.36	65.70	118.07	67.41
2	54.49	55.12	60.87	56.63	69.98	82.50		85.76
3	57.77	58.57	66.77	60.71	80.59	99.31		104.11
4	61.06	62.01	72.67	64.80	91.21	116.12		
5	64.34	65.46	78.57	68.88	101.83			
6	67.62	68.91	84.47	72.96				
7	70.91	72.36	90.37	77.04				
8	74.19	75.81	96.27	81.12				
9	77.47	79.26	102.17	85.20				
10	80.76	82.70		89.29				
11	84.04	86.15		93.37				
12	87.32	89.60		97.45				
13	90.60	93.05		101.53				
14	93.89	96.50						
15	97.17	99.94						
16	100.45	103.39						

(1 point or more) showed significant differences ($p < 0.05$) but one cell had an expected count of < 5 . Collapsing the two middle-age ranges into one group and the four age groups over 45 into one group yielded a trichotomous age group variable. Among those 16–25 years old, 6.2% showed positive DUDIT scores, those 26–45 years old showed 3.7% positive scores and those 45 years or more showed 1.5% positive DUDIT scores ($\chi^2 = 9.61$, d.f. = 2, $p < 0.01$, all cells with expected counts of 5 or more; $n = 1,029$).

Among the respondents who indicated their gender ($n = 1,053$), 48% ($n = 504$) were men and 52% ($n = 549$) were women. Corresponding figures for the entire population aged 16–85 in 2002 from the Swedish Central Bureau of Statistics show 49.4% men and 50.6% women [12]. Among the men, 4.8% ($n = 24$) showed a positive DUDIT score of 1 point or more while 1.6% ($n = 9$) of the women had positive DUDIT scores ($\chi^2 = 8.44$, $p < 0.01$).

T-Scores, Reliability and Factor Structure

T-scores were calculated separately for men and women. The purpose of this analysis was to gain some idea of possible reference values that could indicate drug use both in a case finding and in a public health context. Table 2 shows T-scores in our random sample up to approximately $T = 100$ for men and women of all ages and for men and women aged 16–25, 26–45 and 45 years and over.

A discretionary issue is what level of T-scores should serve as an ad hoc indicator of drug use disorders. Examples of possible choices are scores 1, 2 or 3 standard deviations (SD) from the mean (i.e., $T = 60, 70$ or 80). T-scores at 1 SD from the mean occurred at DUDIT scores of 3 points for men and 1 point for women; T-scores at 2 SD from the mean occurred at DUDIT scores of 6 points for men and 2 points for women, and T-scores at 3 SD from the mean occurred at DUDIT scores of 8 points for men and 3 points for women. In the absence of DUDIT validation in lower prevalence samples, the choice of reference values depends on the purpose of the study at hand. If sensitivity is a high priority, the lower reference values could be chosen. If, on the other hand, specificity is more important, the higher reference values would be more appropriate. In view of the small number of respondents with any drug use at all in the present population sample, the T-score-based reference values we present should in any case be regarded as highly preliminary.

Reliability and factor analyses are questionable in this sample with its highly skewed positive distribution and probable selective dropout of individuals with higher DUDIT scores. However, in order to facilitate standard comparisons with future samples we report the internal consistency according to Cronbach's α (0.93). A valuable addition to reliability indicators for the DUDIT in population samples of this type would be a test-retest reliabil-

ity analysis [13]. The factor analysis is reported in order to allow comparison with an AUDIT population sample and in view of the acceptable Kaiser-Meyer-Olkin coefficient (0.82) indicating sampling adequacy [9]. Principal component extraction with Varimax rotation yielded two factors explaining 73.8% of the total variance. With this solution, however, items 1, 4 and 8 loaded on both factors. An oblique rotation eliminated multiple loading for item 4. According to this solution, the two factors showed a correlation of 0.44 ($n = 1,109$; 33 respondents had total DUDIT scores of 1 or more points). One factor explained 58.9% of the variance and reflected dependence on drugs (items 2, 4–8), and the other explained 14.9% of the variance and reflected drug-related problems (items 1, 3, 9–11). Corresponding AUDIT factors for the general population are alcohol use and alcohol-related problems [14].

Discussion

The DUDIT has good potential for use as a parallel instrument to the AUDIT in screening for drug-related problems in terms of hazardous or harmful use and drug dependence in clinically selected samples. This screening scheme can simplify and streamline information-gathering and documentation of drug problems in target groups where drug users are likely to be found. Pharmacological assays of urine, blood or oral fluid can, of course, complement AUDIT and DUDIT screening in clinical settings [15].

Case Finding Issues

The DUDIT was developed for case identification in groups with high prevalence of drug use, but also in the hope that it would be of value in various care settings with lower prevalence. In our drug user sample the DUDIT accurately predicted *dependence* on multiple drugs with a cut-off score of 25. In lower prevalence samples, cut-off scores for dependence would probably be lower. Cut-off scores for individuals with dependence on different drugs might differ and the factor structure might shift compared to our heterogeneous drug user sample.

Concurrent validity for diagnoses of harmful use or substance abuse in the drug user sample was no better than chance. An obvious explanation is that harmful use or substance abuse was consistently secondary to dependence on one or more drugs. Another possibility is that harmful use/abuse diagnoses are generally problematic, requiring additional conceptual and psychometric work

for clinical use and cautious application in research [16]. Even in a lower prevalence sample establishment of valid cut-off scores for harmful use/abuse diagnoses might thus be questionable.

Combining harmful use/abuse and dependence diagnoses into a single concept reflecting drug-related problems could offer a viable solution. Based on our preliminary T-score analyses from the general population, using reference values 2 SD from the mean for the sake of example, the DUDIT can screen for drug-related problems at scores of 6 for men and 2 for women out of a maximum of 44 points. In comparison, AUDIT screening levels for hazardous or harmful alcohol use are 8 points for men of all ages and 6 for women out of a maximum of 40 points [15]. The T-score-based indicators 2 SD from the mean may appear high in light of the definition of most drug use as illegal. However, sporadic drug use does not necessarily warrant definition as harmful use or dependence. Setting the preliminary indicators at this level might contribute to screening out individuals with drug use that would not lead to diagnosis, an advantage in case finding where scarce resources would make specificity a first priority, rather than sensitivity.

Public Health Survey Issues

Our population sample survey suggests that the DUDIT might be of interest as a useful instrument for mapping drug problems in public health surveys. Although the external validity of our population reference norms could be questioned in light of the selective dropout we identified in the sample from the general population, our level of response (75%) is typical for large, randomly sampled population surveys such as the Swedish Institute of Social Research Level-of-Living Survey 2000 (LNU 2000), with a response rate of 76.1% [J. Fritzell, pers. commun., 2003], and the Statistics Sweden Living Conditions Survey (ULF), with a response rate of 76.7% in 1998 [17].

Prior surveys of drug use in the entire population were not available. However, we were able to compare our figures for the youngest age group, aged 16–25, to Swedish national survey figures for drug use among 9th-grade students 15 or 16 years old [18], and among male conscripts in their late teens and early twenties [19]. Among 9th-grade Swedish boys and girls in 2002, the response rate was 86% due to absenteeism with an additional 8% missing due to refusal by the school principal or teachers, a net response rate of approximately 78%. Among both boys and girls, 8% had tried drugs. A dropout analysis from the 1993 survey of 9th graders showed that

drug use was about twice as high among those who were absent on the day of the survey. Among the conscripts, also tested in 2002, the response rate was 87% due to non-conscription of foreign citizens, chronically ill and handicapped individuals; 89% of these answered the drug questionnaire, giving a net response rate of 81%. Among the conscripts, 3% had used drugs during the past month and 18% had used drugs at some earlier time in their lives. While our net response rate for the general population sample compares quite favourably to the response rates in these standardized surveys, our lower drug use figures compared to the 9th graders may have been influenced by the exclusion of experimentally inclined 15-year-olds from our sample, and by possible increased accuracy of response in the context of a school-based survey. Our higher drug use figure compared to the conscripts might be due to the fact that the DUDIT refers to a period of time over the past year rather than the past month.

*Future Applications*⁵

The present study evaluated the DUDIT in a high prevalence sample of drug users in criminal justice and addiction detoxification settings, and in a random population sample. Although the cut-off score identified in the drug user sample was very high, its satisfactory sensitivity and specificity suggest the DUDIT could be useful as a tool for resource allocation in assessment among heterogeneous samples of drug users within criminal justice and addiction treatment settings. Future studies may yield different cut-off scores depending on preferred drug types. Psychometric properties among groups in other care settings should also be explored. This could lead to establishment of new cut-off scores for specific clinical groups. Such research could also clarify the comparative stability of the DUDIT factor structure.

In clinical settings, we propose that the DUDIT be used in the first step of a four-step assessment procedure. The DUDIT can be completed within approximately 5 min and the AUDIT takes about the same amount of time. This gives quick information on whether the clinician needs to continue the assessment of drug problems. If so, in the second step, a positively screened drug problem is explored in greater detail. An adjunct instrument

to the DUDIT, the self-report DUDIT-E, has been developed for use at this second step [unpubl. data]. The second step can be complemented with a diagnostic interview according to chapter 12 of the SCAN [6], which we adapted and simplified for this study. Once the extent of the drug problem is clear, the procedure continues in its third step with a broader bio-psycho-social person assessment, leading to referral for appropriate treatment. In the fourth step, treatment outcomes are assessed by re-testing. Future clinical research could explore the utility of the DUDIT as a re-test instrument following appropriate treatment.

In the public health context, it may be that the accuracy of DUDIT-based surveys would be greater in standardized yearly surveys among specific groups such as school students and conscripts. In any case, further population surveys are needed for more precise assessments of the frequency of drug use behaviour among different groups in the population.

In conclusion, our study suggests that the DUDIT may, with time, significantly contribute to more effective screening of drug problems in research and clinical settings with groups at risk and that it may be useful for mapping drug use among different groups in public health contexts.

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⁵The DUDIT is a screening instrument in the public domain, but its user-friendly layout is an important part of its integrity and forms the basis for the evaluation of its psychometric properties. We therefore ask that researchers and clinicians interested in trying out the DUDIT contact us for a template for use in Swedish or for translation into other languages. A manual is also available to accompany clinical use of the DUDIT.

DUDIT

Drug Use Disorders Identification Test

Here are a few questions about drugs. Please answer as correctly and honestly as possible by indicating which answer is right for you.

	☐ Man ☐ Woman	Age			
1. How often do you use drugs other than alcohol? (See list of drugs on back side.)	Never ☐	Once a month or less often ☐	2-4 times a month ☐	2-3 times a week ☐	4 times a week or more often ☐
2. Do you use more than one type of drug on the same occasion?	Never ☐	Once a month or less often ☐	2-4 times a month ☐	2-3 times a week ☐	4 times a week or more often ☐
3. How many times do you take drugs on a typical day when you use drugs?	0 ☐	1-2 ☐	3-4 ☐	5-6 ☐	7 or more ☐
4. How often are you influenced heavily by drugs?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
5. Over the past year, have you felt that your longing for drugs was so strong that you could not resist it?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
6. Has it happened, over the past year, that you have not been able to stop taking drugs once you started?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
7. How often over the past year have you taken drugs and then neglected to do something you should have done?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
8. How often over the past year have you needed to take a drug the morning after heavy drug use the day before?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
9. How often over the past year have you had guilt feelings or a bad conscience because you used drugs?	Never ☐	Less often than once a month ☐	Every month ☐	Every week ☐	Daily or almost every day ☐
10. Have you or anyone else been hurt (mentally or physically) because you used drugs?	No ☐	Yes, but not over the past year ☐		Yes, over the past year ☐	
11. Has a relative or a friend, a doctor or a nurse, or anyone else, been worried about your drug use or said to you that you should stop using drugs?	No ☐	Yes, but not over the past year ☐		Yes, over the past year ☐	

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Turn the page to see the list of drugs 

LIST OF DRUGS

(Note! Not alcohol!)

Cannabis	Amphetamines	Cocaine	Opiates	Hallucinogens	Solvents/inhalants	GHB and others
Marijuana	Methamphetamine	Crack	Smoked heroin	Ecstasy	Thinner	GHB
Hash	Phenmetraline	Freebase	Heroin	LSD (Lisergic acid)	Trichlorethylene	Anabolic steroids
Hash oil	Khat	Coca	Opium	Mescaline	Gasoline/petrol	Laughing gas
	Betel nut	leaves		Peyote	Gas	(Halothane)
	Ritaline			PCP, angel dust	Solution	Amyl nitrate
	(Methylphenidate)			(Phencyclidine)	Glue	(Poppers)
				Psilocybin		Anticholinergic compounds
				DMT		
				(Dimethyltryptamine)		

PILLS – MEDICINES

Pills count as drugs when you take

- more of them or take them more often than the doctor has prescribed for you
- pills because you want to have fun, feel good, get "high", or wonder what sort of effect they have on you
- pills that you have received from a relative or a friend
- pills that you have bought on the "black market" or stolen

SLEEPING PILLS/SEDATIVES

Alprazolam	Glutethimide	Rohypnol
Amobarbital	Halcion	Secobarbital
Apodorm	Heminevrin	Sobril
Apozepam	Iktorivil	Sonata
Aprobarbital	Imovane	Stesolid
Butabarbital	Mephobarbital	Stilnoct
Butalbital	Meprobamate	Talbutal
Chloral hydrate	Methaqualone	Temesta
Diazepam	Methohexital	Thiamylal
Dormicum	Mogadon	Thiopental
Ethchlorvynol	Nitrazepam	Triazolam
Fenemal	Oxascand	Xanor
Flunitrazepam	Pentobarbital	Zopiklon
Fluscand	Phenobarbital	

PAINKILLERS

Actiq	Durogesic	OxyNorm
Cocclana-Etyfin	Fentanyl	Panocod
Citodon	Ketodur	Panocod forte
Citodon forte	Ketogan	Paraflex comp
Dexodon	Kodein	Somadril
Depolan	Maxidon	Spasmofen
Dexofen	Metadon	Subutex
Dilaudid	Morfin	Temgesic
Distalgesic	Nobligan	Tiparol
Dolcontin	Norflex	Tradolan
Doleron	Norgesic	Tramadul
Dolotard	Opidol	Treo comp
Doloxene	OxyContin	

Pills do NOT count as drugs if they have been prescribed by a doctor and you take them in the prescribed dosage.

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